

An Act

ENROLLED HOUSE
BILL NO. 3644

By: Stinson and Archer of the
House

and

Hines and Mann of the
Senate

An Act relating to venous thromboembolism; creating the Blake Burgess Act; providing short title; requiring specified facilities to implement certain policies and training regarding venous thromboembolisms; providing certain construction; requiring the State Department of Health to contract for establishment of a statewide venous thromboembolism registry; requiring certain hospitals to report specified information to registry; listing data to be reported; requiring certain entity to collect and report specified data; requiring submission of certain report by the Department; providing requirements for report; limiting use of registry data; amending 63 O.S. 2021, Section 1-890.2, which relates to definitions used in the Continuum of Care and Assisted Living Act; adding definitions; amending 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), which relates to promulgation of rules; requiring certain assessment to include specified screening; requiring assisted living centers to make available certain consumer information pamphlet; providing for noncodification; providing for codification; providing an effective date; and declaring an emergency.

SUBJECT: Venous thromboembolism

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law not to be codified in the Oklahoma Statutes reads as follows:

This act shall be known and may be cited as the "Blake Burgess Act".

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1-630 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. Each hospital with an emergency department and each ambulatory surgical center shall:

1. Develop and implement policies and procedures for the rendering of appropriate medical attention for persons at risk of forming venous thromboembolisms which reflect evidence-based best practices relating to, at a minimum:

- a. assessing patients for risk of venous thromboembolism using a nationally recognized risk assessment tool, and
- b. treatment options for a patient diagnosed with venous thromboembolism; and

2. Train all nonphysician personnel at least annually on the policies and procedures developed under this section. For purposes of this section, the term "nonphysician personnel" means all personnel of the licensed facility working in clinical areas and providing patient care, except those persons licensed as health care practitioners.

B. Nothing in this section shall be construed to allow or require personnel to practice outside the scope of their certification, license, or qualification.

SECTION 3. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1-631 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. The State Department of Health shall contract with the state-designated entity for health information exchange to establish

and maintain a statewide venous thromboembolism registry to ensure that the performance measures required to be submitted under subsection B of this section are maintained and available for use to improve or modify the venous thromboembolism care system, ensure compliance with nationally recognized guidelines, and monitor venous thromboembolism patient outcomes.

B. Beginning July 1, 2027, each hospital with an emergency department shall electronically report annually to the statewide venous thromboembolism registry information containing nationally recognized venous thromboembolism measures and data on the incidence and prevalence of venous thromboembolisms. Such data shall include the following information:

1. The number of venous thromboembolisms identified and diagnosed;
2. The age of the patient;
3. The ZIP code of the patient;
4. The sex of the patient;
5. Whether the patient is a resident of a licensed nursing facility or assisted living center;
6. Whether the venous thromboembolism was fatal;
7. How the diagnosis was made, such as by using imaging modalities; and
8. The treatment that was recommended for the venous thromboembolism.

C. The Department shall require the state-designated entity for health information exchange to collect data from each hospital with an emergency department on the performance measures required under subsection B of this section. The state-designated entity shall provide to the Department regular reports on the data collected.

D. By June 1, 2027, the Department shall electronically submit to the Governor, the President Pro Tempore of the Senate, and the Speaker of the House of Representatives a detailed report on the incidence of venous thromboembolism using inpatient and outpatient data for services provided between the effective date of this act

and June 30, 2027. The report shall provide analyses of all of the following:

1. Age category, initial primary diagnosis and procedure, and secondary diagnoses, readmission rates for inpatients, admission rates for venous thromboembolism for which the patient had an ambulatory surgery procedure, and emergency department visits for venous thromboembolism linked to any previous admission;

2. Whether the venous thromboembolism was present upon admission;

3. The incidence of venous thromboembolism procedures reported on the website of the Department; and

4. The principal payor, the sex of the patient, and the patient's discharge status.

E. The state-designated entity for health information exchange operating the registry shall only use or publish information from the registry for the purposes of advancing medical research or medical education in the interest of reducing morbidity or mortality.

SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is amended to read as follows:

Section 1-890.2. As used in the Continuum of Care and Assisted Living Act:

1. "Assisted living center" means any home or establishment offering, coordinating or providing services to two or more persons who:

- a. are domiciled therein,
- b. are unrelated to the operator,
- c. by choice or functional impairments, need assistance with personal care or nursing supervision,
- d. may need intermittent or unscheduled nursing care,
- e. may need medication assistance, and

f. may need assistance with transfer and/or ambulation;

2. "Board" means the State Board of Health;

3. "Commissioner" means the State Commissioner of Health;

4. "Continuum of care facility" means a home, establishment or institution providing nursing facility services as defined in Section 1-1902 of this title and one or both of the following:

a. assisted living center services as defined in the Continuum of Care and Assisted Living Act, and

b. adult day care center services as defined in Section 1-872 of this title; ~~and~~

5. "Department" means the State Department of Health;

6. "Pulmonary embolism" (PE) means a condition in which part of a clot breaks off and travels to the lungs, possibly causing death; and

7. "Venous thromboembolism" (VTE) means deep vein thrombosis (DVT), which is a blood clot located in a deep vein, usually in the leg or arm. The term can be used to refer to DVT, pulmonary embolism, or both.

SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025, Section 1-890.3), is amended to read as follows:

Section 1-890.3. A. The State Commissioner of Health shall promulgate rules necessary to implement the provisions of the Continuum of Care and Assisted Living Act. Such rules shall include, but shall not be limited to:

1. A uniform comprehensive resident screening instrument to measure the needs and capabilities of residents in all settings and to determine appropriate placements of residents. Such assessment shall include screening for the signs and symptoms of venous thromboembolism (VTE) and techniques for providing an emergency response;

2. Physical plant requirements meeting construction and life safety codes, with provisions accommodating resident privacy and

independence in assisted living centers and in assisted living components of continuum of care facilities based on the variable capabilities of residents;

3. Staffing levels responsive to the variable needs of residents, with provisions for sharing of staff between components in a continuum of care facility;

4. Minimum standards for resident care including, but not limited to, standards pertaining to medical care and administration of medications. Standards pertaining to medication administration shall, at a minimum, require the assisted living center or continuum of care facility to:

- a. provide or arrange qualified staff to administer medications based on the needs of residents,
- b. follow medication administration orders from a qualified health care provider,
- c. ensure that medications are reviewed monthly by a Registered Nurse or pharmacist and quarterly by a consultant pharmacist,
- d. maintain medication administration records and document all medication administration in such records, and
- e. have medication storage and disposal policies;

5. Standards for measuring quality outcomes for residents;

6. Provisions for individualized services chosen by and designed for each resident;

7. Provisions to prohibit facility staff from disclosing a resident's financial information to third parties without written consent of the resident or the designated representative of the resident;

8. Procedures for inspections and investigations of licensed entities to ensure compliance with the Continuum of Care and Assisted Living Act and rules promulgated by the Commissioner;

9. Enumeration of resident rights and responsibilities to be observed by each facility and its staff. Such resident rights shall include the freedom of choice regarding any personal attending physicians and all other providers of medical services and supplies, providing that the minimum standards are met by the provider pursuant to the Continuum of Care and Assisted Living Act, without a financial penalty or fee charged by the assisted living center;

10. Provisions for a surety bond or deposit from each applicant in an amount sufficient to guarantee that obligations to residents will be performed, with provisions for reduction or waiver of the surety bond or deposit when the assets of the applicant or its contracts with other persons are sufficient to reasonably ensure the performance of its obligations;

11. Provisions requiring assisted living centers to make available to residents upon admission a consumer information pamphlet that contains information about venous thromboembolism (VTE) including, but not limited to, risk factors and recognition of the signs and symptoms of VTE;

12. Provisions for the development of a consumer guide or similar resource to be posted on the Internet website of the State Department of Health to assist individuals and families in understanding the services provided by assisted living centers and to compare and select a facility;

~~12.~~ 13. Provisions for posting results of routine inspections and any complaint investigations of each assisted living center on the Internet website of the Department. Such information shall be regularly updated to include the facility's plan of correction and to indicate when a violation of a licensing regulation was corrected by the facility; and

~~13.~~ 14. Provisions requiring execution of a plan of care and a resident service contract with the resident or resident's representative.

B. The nursing care service of a continuum of care facility shall be subject to the requirements, procedures and remedies set out in the Nursing Home Care Act, including provisions relating to resident rights.

C. The adult day care component of a continuum of care facility shall be subject to requirements and procedures specified under the Adult Day Care Act.

SECTION 6. This act shall become effective July 1, 2026.

SECTION 7. It being immediately necessary for the preservation of the public peace, health or safety, an emergency is hereby declared to exist, by reason whereof this act shall take effect and be in full force from and after its passage and approval.

Passed the House of Representatives the 5th day of May, 2026.

Presiding Officer of the House
of Representatives

Passed the Senate the 28th day of April, 2026.

Presiding Officer of the Senate

OFFICE OF THE GOVERNOR

Received by the Office of the Governor this _____
day of _____, 20_____, at _____ o'clock _____ M.
By: _____

Approved by the Governor of the State of Oklahoma this _____
day of _____, 20_____, at _____ o'clock _____ M.

Governor of the State of Oklahoma

OFFICE OF THE SECRETARY OF STATE

Received by the Office of the Secretary of State this _____
day of _____, 20_____, at _____ o'clock _____ M.
By: _____